



Perimenopause in American Women

ISSUES IN AGING-RELATED REPRODUCTIVE HEALTHCARE

What is Perimenopause?

Transitional period in the ~4 years preceding menopause:

Ovarian function declines, fertility decreases, ovulation and menstrual cycles become irregular, and ovaries produce less estrogen and progesterone; menopause is the post-reproductive phase in which the reproductive system has ceased function and ovaries no longer produce eggs.

Most common symptoms:

Hot flashes, night sweats, difficulty sleeping, anxiety and depression, mood disturbances, trouble with memory and concentration, loss of energy and muscle tone, joint stiffness and pain, and genitourinary syndrome, which encompasses issues such as vaginal dryness, pain, chronic UTIs

Typical treatments:

Hormone replacement therapies and other prescription medications, lifestyle modifications, mental healthcare, over-the-counter vitamins and supplements, alternative medicine

Challenges Accessing Aging-Related Reproductive Care

- Inconsistent approaches to counseling and treatment for menopause transition
- Dissatisfaction with provider knowledge and ability to access treatment
 - No standard classification schema or mandated standard of care
 - Inadequate provider education and training
- Scarce research into symptom impacts, treatment efficacy, and postmenopausal health outcomes due to premenopausal intervention, counseling, or treatment
- Social stigma attached to discussing women's reproductive healthcare issues
- Limited patient knowledge about reproductive healthcare
- Lack of geographical or financial access to knowledgeable providers
- Viewed by some as a natural process of aging, separate from reproductive healthcare
 - Results in dismissive treatment, ascribing symptoms to psychological pathology, refusal to trial hormonal therapies, gatekeeping specialty care, and lack of compassion toward women's perimenopausal experience

How does this impact U.S. Society?

- Women make up about 50.5% of the U.S. population (U.S. Census Bureau, 2023)
- Long-term health effects associated with menopause: osteoporosis & cardiovascular disease
- According to our text:
 - 1 in 5 women reported gender-based discrimination in a healthcare setting
 - Doctors are more likely ascribe women's pain to "psychological and emotional, rather than somatic causes."
 - "Studies show undertesting & undertreatment ...for CVD, resulting in higher case fatality rates for women."
- Perimenopausal and menopausal care as public health optimizes healthy aging outcomes for women
- Fear of dismissive medical response, lack of knowledge, or stigma may lead to avoidance of care for reproductive-related aging or seeking care through alternative (unregulated) means
- Symptoms can be physically and mentally distressing and debilitating, & may interfere with social roles, career, and home life
- In the absence of holistic and standardized care, untreated symptoms may drive overuse of unnecessary specialist or emergency services and treatments, leading to harmful polypharmacy

Changes and Reforms to Improve Access and Outcomes

- *Current “solutions” to accessing care and treatment:*

- Use of online-based networks to research and find perimenopause-friendly providers in their area
- Membership-based telehealth concierge services
- Some insurers do cover or partially cover menopausal transition treatment
- Paying out of pocket

- *Future changes:*

- Standardize reproductive phases and establish diagnostic criteria
- Incentivize insurers to mandate aging-related reproductive care and counseling under the general reproductive healthcare umbrella
- Reduce stigma and improve aging-related reproductive healthcare education in the healthcare system
- Continue national longitudinal research studies on treatments, symptoms, and health impacts of early intervention or providing supportive care
- Regulation of herbal supplements and OTC treatments
- Improved access to menopause counseling and hormone replacement therapy
- Increased public messaging and education about aging-related reproductive health